RIVERBEND FAMILY MEDICINE - OFFICE VISIT

Date of service: 04/20/2026 | Provider: Samuel T. Okafor, MD

Chief complaint

Recurrent headaches.

History of present illness

38-year-old woman with recurrent throbbing, usually left-sided headaches for about 2 years, more frequent over the past 6 months. Episodes last 6 to 24 hours and come with nausea and sensitivity to light. She misses 2 to 3 days of work each month. She takes ibuprofen and sumatriptan 50 mg as needed with partial relief.

Past medical history

Mild intermittent asthma. Seasonal allergic rhinitis. Insomnia (2023), resolved.

Medication history

Amitriptyline 10 mg nightly was prescribed in 2023 for insomnia; she took it for about 2 weeks and stopped because of morning grogginess.

Current: sumatriptan 50 mg as needed, albuterol inhaler as needed, cetirizine 10 mg daily.

Family and social history

Mother has migraine. Works as a paralegal. No tobacco. Alcohol 1 to 2 drinks per week.

Examination

BP 118/72, HR 68, BMI 24. Alert, in no distress. Cranial nerves intact. Strength and sensation normal. Gait normal. Fundi normal.

Assessment and plan

Probable migraine without aura. Referral placed to neurology. Continue sumatriptan as needed, no more than 9 doses per month. Start a headache diary.

Preventive options were discussed; she prefers to wait for the neurology visit before starting a daily preventive medication.

Electronically signed: Samuel T. Okafor, MD, 04/20/2026 16:42

LINDEN CREEK NEUROLOGY ASSOCIATES - NEW PATIENT CONSULTATION

Date of service: 06/08/2026 | Provider: Elena M. Castellanos, MD, Neurology | Referred by: S. Okafor, MD

History

Referred for frequent headaches. Attacks are unilateral and pulsating, moderate to severe, worse with activity, with nausea and photophobia. No aura. No focal weakness, vision loss, or speech change. Triggers: poor sleep, skipped meals.

Headache diary reviewed for March through May 2026: 9 migraine days in March, 8 in April, and 10 in May. Sumatriptan used on 6 to 8 days per month.

Examination

Normal mental status, cranial nerves, motor, sensory, coordination, and gait. No papilledema.

Assessment

Episodic migraine without aura (ICD-10 G43.009). Frequency supports daily preventive treatment.

Preventive treatment discussion

Propranolol was considered but not started because of her history of asthma. Topiramate reviewed, including paresthesia, cognitive slowing, nausea, and the need for reliable contraception.

Plan

Start topiramate 25 mg nightly today, 06/08/2026, increasing by 25 mg each week to a target of 50 mg twice daily.

Continue sumatriptan 50 mg as needed. Continue headache diary. Return in 4 weeks.

Electronically signed: Elena M. Castellanos, MD, Neurology, 06/08/2026 11:05

LINDEN CREEK NEUROLOGY ASSOCIATES - FOLLOW-UP VISIT

Date of service: 07/06/2026 | Provider: Elena M. Castellanos, MD, Neurology

Interval history

Four weeks on topiramate. She reached 50 mg twice daily on 06/29/2026. She reports nausea most mornings and intermittent dizziness since the dose reached 75 mg per day. No paresthesia. No word-finding difficulty. Diary shows 7 migraine days in June.

Assessment and plan

Migraine frequency not yet improved; adverse effects present. Dose adjusted to 25 mg in the morning and 50 mg at night, taken with food. Plan to continue so that she completes an adequate trial. Return in 4 weeks.

Electronically signed: Elena M. Castellanos, MD, Neurology, 07/06/2026 09:48

LINDEN CREEK NEUROLOGY ASSOCIATES - TELEPHONE ENCOUNTER

Date: 07/20/2026 | Taken by: Nina Park, RN

Patient called. Nausea continues on the adjusted dose and she feels dizzy when standing up quickly. No falls. Advised to keep taking the adjusted dose with food, drink fluids, and keep the 08/03/2026 visit. Message routed to Dr. Castellanos.

LINDEN CREEK NEUROLOGY ASSOCIATES - FOLLOW-UP VISIT

Date of service: 08/03/2026 | Provider: Elena M. Castellanos, MD, Neurology

Interval history

Patient completed an eight-week trial of topiramate, 06/08/2026 to 08/03/2026. Treatment was discontinued today due to significant nausea and dizziness, which were not tolerable despite dose adjustment. Migraine frequency was unchanged, with 8 migraine days in July.

Examination

BP 112/70, HR 72. Neurologic examination unchanged and nonfocal.

Assessment and plan

Episodic migraine without aura, inadequately controlled. Topiramate stopped because of intolerance. A beta blocker is avoided because of asthma. Recommend a CGRP monoclonal antibody. Will request prior authorization for erenumab-aooe 70 mg once monthly. Continue sumatriptan as needed. Return in 6 weeks.

Electronically signed: Elena M. Castellanos, MD, Neurology, 08/03/2026 10:21

LINDEN CREEK NEUROLOGY ASSOCIATES - FOLLOW-UP VISIT

Date of service: 09/14/2026 | Provider: Elena M. Castellanos, MD, Neurology

Interval history

Nausea and dizziness resolved within one week of stopping topiramate. Headache diary for June through August 2026: 7 migraine days in June, 8 in July, and 9 in August. She missed 3 days of work in August.

Plan

Prior authorization request for erenumab-aooe to be submitted 09/15/2026. Injection teaching completed today.

Medication list as of 09/14/2026

- Erenumab-aooe 70 mg subcutaneously once monthly - ordered by Elena M. Castellanos, MD (Neurology); pending prior authorization.
- Sumatriptan 50 mg by mouth as needed for migraine, maximum 9 doses per month.
- Albuterol inhaler, 2 puffs as needed.
- Cetirizine 10 mg daily.
- Topiramate - discontinued 08/03/2026 (nausea, dizziness).

Electronically signed: Elena M. Castellanos, MD, Neurology, 09/14/2026 14:30